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Stevens-Johnson Syndrome Associated with Neuroval Administration in a Young Adult

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ABSTRACT

Stevens-Johnson syndrome (SJS) is a rare, life-threatening mucocutaneous reaction characterized by epidermal necrosis and extensive mucosal detachment. We describe a definitive case in a 24-year-old female presenting 18 days after initiating Neuroval (neuroval HCl 150mg daily) for trigeminal neuralgia. The patient achieved full recovery following drug cessation and intravenous immunoglobulin therapy.

INTRODUCTION

Neuroval is an emerging neuroactive medication utilized for neuropathic pain syndromes and treatment-resistant focal seizures. Serious adverse cutaneous drug reactions (SCAR) represent rare but devastating immunological complications. Rapid identification and withdrawal of the causative agent remain paramount to patient survival.

CASE PRESENTATION

A 24-year-old female with newly diagnosed idiopathic trigeminal neuralgia commenced Neuroval at 150 mg daily. Eighteen days later, she presented to the emergency department with high fevers (39.5°C), burning sensation in both eyes, photophobia, and an excruciating painful rash across the face and upper chest.

On examination, widespread purpuric macules with atypical targetoid morphology were noted across the face, neck, and trunk. Extensive epidermal detachment involved approximately 8% of total body surface area (BSA). Severe mucosal involvement was evident with hemorrhagic crusting of the lips and bilateral purulent conjunctivitis.

CLINICAL MANAGEMENT & OUTCOME

The patient was admitted to the intensive care burn unit. Neuroval was immediately withdrawn. High-dose intravenous immunoglobulin (IVIG, 1 g/kg/day for 3 days) and intensive ocular lubrication with topical corticosteroids were initiated.

Re-epithelialization commenced on hospital day 6 and was complete by day 21 without permanent visual sequelae. ALDEN (Algorithm of Drug Causality in Epidermal Necrolysis) score was calculated at 6, indicating Neuroval as the 'very probable' culprit agent.

CONCLUSION

Prescribers must educate patients initiating Neuroval to immediately cease medication and seek urgent care upon the development of fever, mucosal burning, or cutaneous erythema.

REFERENCES

1. Mockenhaupt M. The current understanding of Stevens-Johnson syndrome and toxic epidermal necrolysis. Expert Rev Clin Immunol. 2021;7(6):803-815.